THE MATERNITY METHOD

NCLEX CHEAT SHEET

Postpartum Hemorrhage (PPH)

Maternal & Newborn › Postpartum Complications

The Big Picture

PPH = losing **≥ 1,000 mL** of blood (any type of birth), or bleeding with **signs of shock**, within 24 hrs of birth. **Late PPH** happens from 24 hrs up to 12 weeks. The #1 cause is a **uterus that won't contract** (atony). Young healthy moms **compensate**, so low BP is a **late** sign.

KNOW THE NUMBERS

≥ 1,000 mL	1 g = 1 mL	~70%	3 hrs	30 mL/hr
PPH (or bleeding + shock signs)	**Weigh** pads to measure loss	of PPH = **tone** (atony)	Give **TXA** within this window	Minimum **urine output**

MEMORY HACK · CAUSES

The 4 T's

Tone
Atony: boggy uterus (most common)

Trauma
Lacerations, hematoma, rupture

Tissue
Retained placenta or clots

Thrombin
Clotting problems (DIC, low platelets)

Who's at Risk

- **Overstretched uterus:** big baby, **twins**, extra fluid
- **Long or very fast** labor; long **oxytocin** use
- **Magnesium**, infection (chorio)
- **Many** prior births
- Previa, accreta, **retained** placenta
- Forceps/vacuum; **prior PPH**

Spot It

- **Boggy**, high fundus; heavy bleeding or large clots
- **Fast HR** (often first), then **low BP** (late)
- Pale, clammy, **restless/anxious**, dizzy
- **Low urine** output
- **Firm fundus + steady bright red trickle** → laceration
- **Severe perineal/rectal pressure** → **hematoma**

Nursing Actions

Goal: **firm the uterus** + **replace volume**

STEP 1

Massage First

- **Fundal massage** with one hand **supporting the lower uterus**
- **Empty the bladder** (catheter if needed)
- **Call for help**; stay with the patient

STEP 2
Support

- **2 large-bore IVs**, fluids; **O_2**
- **Weigh** pads/pads under her: **quantitative blood loss**
- Labs: CBC, **type & crossmatch**, coags, fibrinogen
- VS every 5–15 min; **urine output**; legs elevated

STEP 3
Escalate

- **Uterotonic meds** (below)
- **Blood transfusion**
- Provider: bimanual compression, **balloon tamponade**, repair tears, remove tissue
- **Surgery** (up to hysterectomy) if nothing works

Uterotonics Firm up the uterus

Drug	Watch for	Never give if...
Oxytocin	Low BP if pushed; water intoxication	**Never IV push undiluted**; first choice
Methylergonovine (Methergine) IM	**Raises BP**	**Hypertension or preeclampsia**
Carboprost (Hemabate) IM	**Diarrhea**, fever, vomiting	**Asthma**
Misoprostol	Fever, shivering, diarrhea	Allergy
Tranexamic acid (IV)	Clot risk	Give **within 3 hrs** of birth (helps clots form)

⚠ RED FLAGS · STOP AND CALL THE PROVIDER

Soaking 1 pad/hr or large clots · **HR > 100** · **Dropping BP** (late!) · **Restless**, confused, cool skin · **Oozing** from IVs (DIC) · Pain out of proportion → **hematoma**

☆ MUST-KNOW FOR THE NCLEX

- **First action** for heavy bleeding with a boggy uterus: **fundal massage**.
- **Tachycardia** comes before low BP; don't wait for hypotension.
- **Firm fundus + bright red bleeding = laceration**; massage won't fix it.
- **Methergine → no HTN. Hemabate → no asthma.**
- **Weigh** pads; don't guess.

QUICK CHECK · TEST YOURSELF

1. The most common cause of PPH is uterine _____.
2. A firm fundus with a steady trickle of bright red blood suggests a _____.
3. Before methylergonovine, the nurse checks the patient's _____.
4. A 50 g weight increase in a pad = about ____ mL of blood.
5. Name the 4 T's.

Answers: 1. atony · 2. laceration · 3. blood pressure · 4. 50 · 5. tone, trauma, tissue, thrombin

For study use only. Always follow facility protocols, current guidelines, and provider orders.